

firearm ownership and storage, poison and medication security, fall prevention, and emergency preparedness, adapted primarily from the National Fire Protection Association Home Fire Safety Survey. Demographic items were added by an October 2024 amendment and an item on interest in a complimentary gun lock by a January 2025 amendment; these analyses were therefore limited to defined subgroups enrolled later in the Study rather than random subsets. Among patients enrolled after the gun lock amendment, all were offered a gun lock regardless of response. Categorical responses were summarized with counts and percentages, and associations between demographics and gun lock interest were assessed with Fisher exact, chi-square, and Wilcoxon rank-sum tests, with adjusted odds ratios estimated by logistic regression.

Results: Among 1,000 respondents, home safety deficits were common across multiple domains. For fire safety, 87% reported a smoke detector on every level but only 49% in every bedroom, 60% had a carbon monoxide detector, and 63% had a household emergency plan. For firearms, 274 (27.4%) reported one or more in the home, 204 of 501 answering (41%) reported easy access to a firearm, and among owners 14.7% (95% CI 10.7–19.4%) reported a firearm that was not securely stored. Household poisons and prescription medications were stored insecurely by 31% and 36% of respondents, respectively, and only 9.2% always wore a helmet when cycling. Among the 180 participants enrolled after the gun lock amendment, 104 responded to the interest item, of whom 37 (35.6%; 95% CI 26.4–45.6%) expressed interest, including 14 of 43 firearm owners (32.6%) in this subgroup. No demographic characteristic was significantly associated with interest in adjusted analyses, including odds ratios of 1.46 (95% CI 0.64–3.41) for female versus male and 1.30 (95% CI 0.57–3.05) for non-owners versus owners.

Conclusion: Adult ED patients demonstrated widespread home safety gaps spanning fire, firearm, poison, medication, and fall-prevention domains, underscoring substantial unmet injury prevention needs. Among patients offered a complimentary gun lock, over a third expressed interest, including nearly a third of firearm owners, with no concentration in any demographic subgroup, supporting universal rather than demographically targeted distribution. These findings support an ED visit as a practical and broadly acceptable opportunity for home safety screening and interventions for harm reduction.

Disclosures: No, authors do not have interests to disclose

597 Novel Standardized STI Screening Implementation in Colorado Emergency Departments



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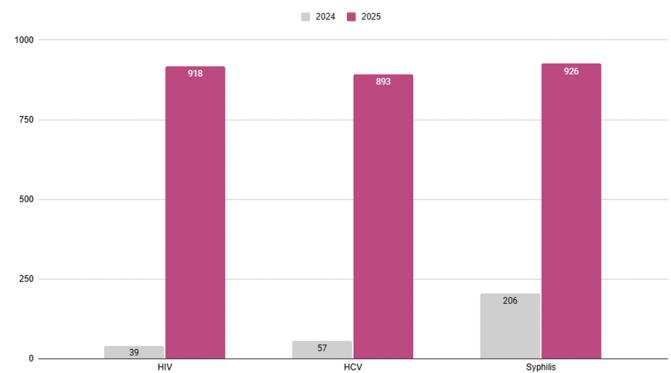
Study Objectives: St. Anthony North (SAN) Emergency Department (ED) is an urban facility located in the Denver metropolitan area. In 2022, the Denver metropolitan area accounted for 62.4% of all new HIV diagnoses with an average CD4 count of 399, indicating a need for improved early detection Methods: (CDPHE, 2022). As a result SAN implemented screening to scale-up testing for HIV, HCV and Syphilis. The program received braided grant funding and utilized evidence based techniques for successful implementation to increase testing uptake. This analysis examines the Methods: of the program and its effectiveness to increase HIV, HCV, Syphilis screening from 2024 to 2025 in Colorado. The Objective: of this Study is to show viable evidence for similar programs in Colorado and their ability to improve population health through early detection and prevention of HIV, HCV, and Syphilis.

Methods: FOCUS was launched on January 1st, 2025 and utilized U.S Preventive Services Task Force (USPSTF), CDPHE and the Centers for Disease Control (CDC) guidelines to inform screening eligibility. Patients were eligible for screening based on age (18–64), blood draw planned as part of their ED visit, and who had not received a test within a certain time period, determined through electronic health record (EHR) documentation. Providers were introduced to integrated screening workflows, which were implemented along existing ED practices to incorporate counseling and testing for eligible patients. Additionally, the program included dedicated program staff to establish robust workflows for testing, diagnosis, assessment of patients needing preventative services, and successful linkage to treatment. Continuous quality improvement informed developing a novel process for the region, 'ED patient counseling', which drastically increased screening rates as a result of improved patient rapport and regulatory compliance. A retrospective data analysis was utilized to assess before and after Results, comparing 2024 (pre-implementation) HIV, HCV, Syphilis screening, positivity, and linkage to care rates to 2025 (post-implementation) rates. The Study reviews all patients who received an HIV, HCV, and/or Syphilis test during their visit at St. Anthony North Emergency Department. Sample size includes a total of 111,848 patients.

Conclusion: The implementation of standardized HIV, HCV, Syphilis screening program at St. Anthony North ED significantly expanded STI testing, increasing total

tests across HIV, HCV, and Syphilis an impressive 84% from 2024 to 2025. While the overall positivity rate predictably decreased from 4.30% (targeted) to 1.38% (non-targeted), the program achieved a crucial public health goal: a substantial increase in the absolute number of detected infections for all three STIs (Syphilis: 11 to 25; HIV: 1 to 4; HCV: 1 to 9). This demonstrates program effectiveness in identifying previously undiagnosed cases within a broader population, thereby improving early detection and facilitating a greater number of patients to be linked to care and preventative services. Additionally, 2025 saw a 78.0% linkage to care rate compared to 2024 of 33.3%. These findings provide compelling evidence for the efficacy of integrated, non-targeted STI screening programs in ED settings to enhance population health and combat STI prevalence in urban areas like the Denver metropolitan region.

HIV, HCV, Syphilis Tests 2024 vs. 2025



Disclosures: Yes, authors have interests to disclose

Disclosure: Grant Support. Gilead Sciences Inc

Disclosure: Grant Support. Colorado Department of Public Health and Environment

598 Before the Fall: Connecting High-Risk Adults to Prevention Before Emergency Department Visits



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Objectives: Unintentional falls drive approximately 8.9 million emergency department (ED) visits annually in the United States. Validated prevention exists; consistent delivery does not – saddling EDs saddled with the downstream burden. To address this, our emergency department, trauma program, and school of nursing designed and piloted an upstream community-based fall prevention program. This model aimed to reach high-risk, community-dwelling adults outside routine care through walk-in events. We aimed to characterize participants' fall risk and prior fall-prevention engagement, assess longitudinal changes in prevention behavior and fall outcomes, and evaluate the feasibility of recruitment, retention, and follow-up to support a future hybrid type-1 effectiveness-implementation trial.

Methods: We conducted a prospective observational pilot Study at five walk-in community fall prevention events. Events were held from 2023 to 2026, spanning both rural and metro areas. They were staffed by trained volunteers and nursing students, with support from the emergency department and nursing faculty. All adults aged 18 and older were eligible and asked to enroll. Consent was obtained to retain event-day assessment data, access electronic health records, and complete longitudinal follow-up surveys at 6 months. Regardless of consent status, all attendees received validated screenings, including the CDC's Stopping Elderly Accidents, Deaths and Injuries (STEADI) fall-risk screening and the Short Falls Efficacy Scale-International (Short FES-I) concern of falling screening, followed by brief motivational interviewing, individualized prevention education, and local resources. Participants were eligible for follow-up if they had reached their six-month follow-up window at the time of analysis. Chart review confirmed pre- and post-event fall prevention engagement, risk factors, and fall outcomes. STEADI scores ≥ 4 defined high fall risk; Short FES-I scores were categorized as low (7–8), moderate (9–13), or high concern (14–28).

Results: Of 193 attendees across five events, 64 adults (mean age 73.6 ± 9.8 years; 75.0% female) enrolled. At enrollment, 42/63 (66.7%) screened as high fall risk, and

35/64 (54.7%) reported a fall in the prior year, including 32/42 (76.2%) of high-risk participants. 37/64 (57.8%) reported no prior PCP fall-prevention discussion. Additionally, 56/64 (87.5%) had never attended a fall-prevention class, and 55/64 (85.9%) reported moderate-to-high concern about falling. Of 52 eligible participants, 28/52 (53.8%) completed the six-month follow-up. Among paired respondents, fall-prevention class attendance increased from 4/26 (15.4%) at baseline to 12/26 (46.2%) by 6 months. Chart-documented post-event falls occurred in 15/52 (28.8%). Falls were more common in high-risk participants than those below the high-risk threshold (13/33, 39.4%, vs 1/18, 5.6%; RR 7.09, 95% CI 1.01–49.9), a wide confidence interval reflecting sparse events in the below-threshold group. Fall-, balance-, or dizziness-related ED or urgent care visits occurred in 5/52 (9.6%).

Conclusion: Two-thirds of community-dwelling participants screened high-risk. Despite nearly all expressing concern about falling, most reported not having a fall-prevention discussion with their PCP. Post-event falls occurred at seven times the rate among high-risk participants when compared to low-risk participants. Fall-prevention class attendance more than doubled after the event. These findings suggest that our community-based fall-prevention model identified and reached adults with substantial fall risk, high concern about falling, and limited prior prevention engagement, while delivering meaningful evidence-based prevention. This pilot provided feasibility, targeting, and outcome-rate estimates to power a future hybrid type 1 effectiveness-implementation trial assessing whether upstream community-based fall prevention can serve as a bridge to reduce fall-related ED visits and downstream acute care burden.

Disclosures: No, authors do not have interests to disclose

599 Parks for Gun Violence Prevention: A Constructivist Grounded Theory



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Objective: Emergency medicine clinicians treat patients injured by gun violence in emergency departments throughout the United States. The CDC has adopted a prevention framework that seeks to address individual, relationship, community and societal factors contributing to gun violence. Community factors include the built environment, such as buildings, streets, and green space. Parks are another aspect of the built environment. Whether public parks may play a role in gun violence prevention is unknown. We sought to construct a theory of how parks may play a role in gun violence prevention from the insights and lived experiences of park users in three Los Angeles communities historically affected by high rates of gun violence.

Methods: Early in our research process, we formed a community advisory board (CAB) consisting of three members from our study communities. Our CAB collaborated on all aspects of the research process and provided invaluable insider knowledge of our Study communities. After discussion with our CAB and key stakeholders, we selected three large public parks as our primary Study sites. We collected data using two different qualitative methodologies. First, we conducted a six-month period of field work using the ethnographic technique of participation observation in the parks and surrounding communities. Next, we conducted intensive interviews with 12 park users. Using constructivist grounded theory methodology, we constructed a grounded theory from this data, Parks as “Safe Havens” From Gun Violence.

Results: Participants viewed parks as community “safe havens” that helped prevent gun violence in the surrounding neighborhoods. Two aspects of parks helped prevent gun violence. A shared community norm of protecting children led people to avoid violence when children were present and empowered others to confront threats of violence. Trusted messengers – volunteers, park staff, and coaches helping with recreational activities – earned respect from other park users. They opened channels of communication that prevented situations from escalating into violence and became mentors capable of dissuading youth from joining local gangs, thought to be drivers of gun violence. Together, the presence of children and trusted messengers turned parks into “safe havens” that reduced gun violence in their communities.

Conclusion: some public parks are already playing meaningful roles in gun violence prevention by serving as safe spaces within their communities. Other places have leveraged similar elements present in our study’s parks to house gun violence prevention efforts. For example, hospitals are generally considered to be safe spaces in communities. Many hospitals have utilized this designation to host hospital-violence prevention programs (HVIPs), which use trusted messengers to reach patients injured

by gun violence in hopes of breaking cycles of violence. Like hospitals, our parks were considered safe spaces and featured trusted messengers. They may be excellent places to host prevention programs similar to HVIPs, thus offering potential to reduce the number of patients in emergency departments injured from gun violence.

Disclosures: No, authors do not have interests to disclose

600 Where Fatal Traumatic Brain Injury Occurs Changed: A 25-Year National Analysis of Death Location



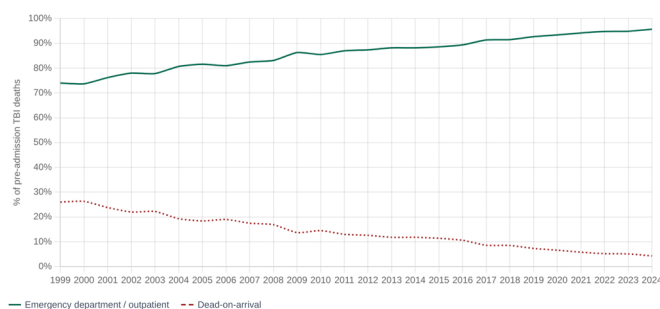
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Objective: Traumatic brain injury is among the most time-critical emergency diagnoses, and the place of death marks how far a patient progressed along the acute trauma-care pathway. This Study characterized national trends in the location of fatal traumatic head injury in the United States from 1999 to 2024, focusing on the shifting balance between deaths occurring dead on arrival and deaths occurring in the emergency department.

Methods: A retrospective serial cross-sectional analysis of the CDC WONDER Multiple Cause of Death database (1999–2024) was performed. Deaths involving traumatic brain injury were identified by ICD-10 codes for skull and facial fracture, intracranial injury, and head-injury sequelae (S01–S09, T90) appearing on the death certificate, then classified by documented place of death. Among deaths occurring before hospital admission (dead on arrival or emergency department/outpatient), the proportion in each setting was calculated annually, alongside the emergency-department-to-dead-on-arrival ratio. Temporal trends in each proportion were summarized as the annual percent change from log-linear regression of the proportion on calendar year, with 95% confidence intervals. Data for 2024 are provisional.

Results: Among pre-admission traumatic brain injury deaths, the emergency department share rose from 74% to 96% (annual percent change +1.02%; 95% CI +0.95 to +1.10; $p < 0.001$), while the dead-on-arrival share fell from 26% to 4% (annual percent change –6.70%; 95% CI –7.17 to –6.23; $p < 0.001$). The emergency-department-to-dead-on-arrival ratio widened from roughly 3:1 to 22:1. Dead-on-arrival deaths fell 88% in absolute number (2,259 to 280) and from 9.7% to 1.0% of all medical-facility traumatic brain injury deaths (annual percent change –8.25%; 95% CI –8.64 to –7.87; $p < 0.001$), while emergency department death volume remained stable (approximately 6,400 annually). Combined pre-admission deaths declined 25% even as inpatient deaths rose 40%.

Conclusion: Over a quarter century, the emergency department has become the setting where nearly all pre-admission traumatic brain injury deaths occur, as arrival without signs of life has become increasingly uncommon. The marked decline in dead-on-arrival deaths was not accompanied by increased emergency department death volume; instead, combined pre-admission deaths fell while inpatient deaths rose. These findings argue against simple reclassification of deaths from the dead-on-arrival category into the emergency department and suggest a broader shift in where fatal traumatic brain injury occurs along the trauma-care pathway. Further work should examine injury severity, classification practices, and trauma-system evolution underlying this change.



Disclosures: No, authors do not have interests to disclose